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Meaningful Engagement: Exploring Principles and Actions to Build Client and Caregiver Engagement Within a Community-Based Dual Diagnosis Service

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ABSTRACT

Background: Engagement of individuals with an intellectual disability and their caregivers in health care has been highlighted as an essential factor in developing services that better meet client needs; however, few studies have focused on the engagement of individuals with a dual diagnosis (intellectual disability co-occurring with a mental health illness and/or challenging behaviour) in health care.

Methods: This qualitative participatory action study engaged clients and their caregivers in exploring principles and actions that can be applied to build active and meaningful engagement within a specialised community-based mental health service for adults with a dual diagnosis.

Results: The four themes that emerged from the data—Collaboration, Communication, Empowering Environment, and Tailored Service—offered a way to conceptualise client and caregiver engagement.

Conclusion: This study underscores the importance of building a culture of service delivery that values, encourages, and supports client and caregiver engagement. Practical strategies for increasing engagement are provided.

1 | Introduction

Individuals with an intellectual disability have a higher risk of experiencing mental health problems than those without an intellectual disability (Lunsky et al. 2013; Mazza et al. 2020). Accessing high-quality mental health services is a prominent concern as almost half (48.6%) of the adults in Ontario with an intellectual disability also have a mental illness (Lunsky et al. 2013). In Canada, dual diagnosis is defined as an intellectual disability coexisting with a mental health problem and/or challenging behaviour (Lunsky et al. 2013; McCreary and Jones 2013; Nicholas et al. 2017; Weiss et al. 2009).

Research conducted in the past two decades has consistently noted challenges faced by persons with a dual diagnosis in finding accessible and effective mental health services (e.g., Lunsky et al. 2007; Nicholas et al. 2017; Spassiani et al. 2017; Weiss and Lunsky 2010). Some identified gaps and barriers to accessing high-quality mental health services have included health professionals who lack expertise in dual diagnosis (Ee et al. 2022; Man and Kangas 2020; Whittle et al. 2018), stigma and/or disrespect due to negative attitudes (Ee et al. 2022; Spassiani et al. 2017), negative experiences with and limited trust in professional help (Weiss and Lunsky 2010), communication and/or cognitive limitations of the individual with a

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Summary

- This research adds important knowledge by providing insights and possibilities for engaging clients with a dual diagnosis and their caregivers in their health care provided through a community-based service.
- Clients and their caregivers expressed their desire to be engaged in the client's care. The four areas emerging from this research – Collaboration, Communication, Empowering Environment, and Tailored Service – offer a way to describe principles of meaningful engagement within a community-based dual diagnosis service.
- This study identifies a collection of practical engagement strategies for clients and their caregivers within one community-based dual diagnosis service in Ontario. These strategies can help similar facilities effectively engage, empower, and provide a voice to clients and their caregivers, and positively impact the health care received.

dual diagnosis (Lunsky et al. 2013), poor communication and collaboration between services and/or clinicians (Griffiths et al. 2002; Man and Kangas 2020; Whittle et al. 2018), and diagnostic overshadowing (Weiss et al. 2009; Whittle et al. 2018).

In general, client and caregiver engagement has been described as a critical and prominent strategy in the transformation and redesign of health care (Bombard et al. 2018; Carman et al. 2013; Majid and Gagliardi 2019a, 2019b). Although several engagement frameworks exist that describe levels of client and caregiver engagement within health care, there is a lack of clarity around meaningful engagement as a concept, and who, how, and where to engage an individual (Barello et al. 2012; Ortiz Halabi et al. 2020; Majid and Gagliardi 2019a, 2019b). The inconsistency and variation in terminology, frameworks, and overall lack of conceptual clarity present challenges for health care organisations when attempting to operationalise and evaluate client and caregiver engagement (Barello et al. 2012; Ortiz Halabi et al. 2020; Majid and Gagliardi 2019a, 2019b).

Although limited, previous research on the engagement of individuals with a dual diagnosis has mainly focused on the individual level of health services, service planning and delivery, and policy development, across both inpatient and community contexts. At the individual level, two studies in the United Kingdom (UK) explored the experiences of individuals with a dual diagnosis in the Care Programme Approach process, highlighting similar strategies for increasing client engagement, such as preparing the client before the meeting, providing choice for meeting location and time, ensuring accessible information is used, and increasing client decision-making in the meetings (Hall et al. 2009; Rao and Markar 2006). Another UK study explored the involvement of individuals with a dual diagnosis in both inpatient and community mental health settings, identifying challenges to client participation that included moral and religious beliefs, policy impacts, and an organisational culture that does

not value engagement (Chaplin et al. 2009). At the programme level, a UK study consulted service users, carers, and families to revitalise a specialised inpatient mental health assessment and treatment unit, noting the importance of transparency and honesty with stakeholders, and using a collaborative approach (Chaplin et al. 2011).

Previous research had also explored engagement-related topics such as service user perspectives, assessment and treatment, decision-making, policy development, and information accessibility. For example, Dew et al. (2018) reviewed Australian mental health policy and found inadequate attention to the needs of the dual diagnosis population. These authors recommended the involvement of people with a dual diagnosis, along with caregivers and health providers, in a multifaceted approach to inclusive policy development.

Although the literature has highlighted the engagement of individuals with an intellectual disability and their caregivers as a critical factor in improving accessibility and/or quality of care (e.g., Dew et al. 2018; Lunsky et al. 2013), there continues to be a paucity of studies focused on the engagement of individuals with a dual diagnosis, with research often having a focus on one aspect of engagement, rather than a comprehensive overview of strategies. Without evidence for how to address the diverse and complex needs of individuals with a dual diagnosis at various levels, clinicians may struggle to meaningfully engage these individuals as partners in health care.

This study aimed to address this knowledge gap by focusing on a specialised community-based dual diagnosis service to develop a cohesive list of practical engagement strategies at both the individual and programme levels. This study was guided by the central research question: What principles and actions can be applied to build active and meaningful client and caregiver engagement within a specialised community-based mental health service for adults with a dual diagnosis, at both individual and programme-level services?

2 | Methods

2.1 | Study Design

This qualitative study used a participatory action research (PAR) approach guided by a transformative paradigm (Mertens 2010; Mertens 2017) and drew from the five inclusive research principles of Walmsley and Johnson (2003). Inclusive research is a term used to describe a research process that actively involves individuals with an intellectual disability collaboratively within the research, rather than as participants who provide only data (Frankena et al. 2015; Schwartz et al. 2020; Walmsley and Johnson 2003). These inclusive research principles were used as a secondary strategy to structure and support the PAR methodology and guide the engagement of individuals with a dual diagnosis.

The conceptualisation of engagement drew from the Multidimensional Framework for Patient and Family Engagement in Health and Health Care (MFPFE-HHC), which

frames client and caregiver engagement through three aspects: engagement level, engagement form, and factors impacting an individual's willingness and ability to engage (Carman et al. 2013). This study focused on the MFPFE-HHC engagement levels of 'direct care' and 'organisational design and governance', using the terminology of 'individual level' (direct health care provision to a client) and 'programme level' (the clinical service/programme) services. The first author led the study, but all phases were guided and supported by a Project Advisory Group (PAG). The PAG assumed an advisory role as described in Bigby et al. (2014).

2.2 | Participants

Participants were recruited from a multidisciplinary specialised community-based mental health service located in Ontario, Canada, that provides consultation, assessment, and goal-based treatment for individuals 16 years of age and older with a dual

diagnosis (intellectual disability coexisting with a mental health problem and/or challenging behaviour). Participant categories (see Table 1) included client, informal caregiver, formal caregiver, and clinical and non-clinical staff.

2.3 | Recruitment and Consent

Recruitment occurred in three phases in order to identify participants for the PAG, focus groups, and feedback sessions. Due to her dual role as researcher and occupational therapist in the study setting, the first author drafted the inclusion criteria identified in Table 1, but tasked study-setting clinicians with identifying eligible participants. Either these clinicians or the recruitment assistant obtained the participant permission to be contacted about the study. This aided in the mitigation of potential bias and/or coercion and increased study credibility (Creswell and Creswell 2018; Lincoln and Guba 1985).

TABLE 1 | Inclusion criteria for each participant category in the participatory action research study.

Participant category	Inclusion criteria
Project Advisory Group	• Client or a formal/informal caregiver for a client. • Knowledge and/or experience with the clinical service. • Knowledge and/or experience with the mental health and/or developmental service systems. • Confident and willing to share opinions and ideas. • Meet the inclusion criteria in respective category of Client and Caregiver (formal or informal).
Client	• Adult receiving services from a specialised community-based dual diagnosis service. • Receptive and expressive communication skills that allow contributions to group discussions. • Adequate computer and Internet to participate in virtual groups and/or telephone suitable for an interview. • Ability to communicate over computer and/or telephone, with or without a support person.
Caregiver (informal and formal)	• Provide regular support to clients (i.e., regularly attends client appointments and provides support to clients in-person or virtually at least once per week). • Appropriate computer and Internet access to allow participation in virtual groups and/or telephone suitable for an interview. • Ability to communicate over computer and/or telephone. • Informal: Parent, grandparent, sibling, aunt, uncle, cousin, friend, and neighbour of the client • Formal: Developmental Service Agency front-line support worker or group home worker.
Additional participant requirements	• No relationship between clients (i.e., only one client per family and/or home). • No relationship between caregivers (i.e., one caregiver per family). • Caregivers can support clients with all levels of intellectual disability (i.e., mild, moderate, severe, profound). • Aimed for a diverse sample mixed along the lines of gender, age, nature of disability and geographic area.
Clinical and non-clinical staff	• Recruitment: Clinical staff with 0.8 FTE to 1.0 FTE and Administrative Assistant from the study setting. • Feedback session: Clinical and non-clinical staff.

Abbreviation: FTE = full-time equivalent.

For the PAG and focus groups, client and caregiver recruitment involved a combined letter of information and consent form (LOI/CF) that was developed for each participant category (i.e., PAG clients, PAG caregivers, focus group clients, focus group caregivers). The client LOI/CFs were written in accessible language (Hoole and Morgan 2011; Martin et al. 2016). Maximum variation purposeful sampling (Patton 2015) was used to select participants for both the PAG and focus groups. For the latter, a de-identified list of potentially eligible participants was created to mitigate the potential for bias and/or coercion. Online/telephone screening interviews were completed by the first author to obtain verbal informed consent. All clients had a mental health diagnosis and a neurodevelopmental disorder, with compromised cognitive and adaptive functioning, and had the capacity to provide consent.

Participants in client and caregiver feedback sessions were recruited from focus group participants by email and phone outreach. Clinicians were recruited for feedback sessions through email, followed by a screening interview to obtain verbal informed consent. See Table 2 for participant demographics.

2.4 | Procedure

All meetings, focus groups, individual interviews, and feedback sessions were facilitated by the first author and occurred over the Microsoft Teams videoconferencing program or telephone as required by COVID protocols. Individual interviews were offered to informants who were not comfortable with the group format or did not have appropriate technology to accommodate the online format. The first author drafted client and caregiver focus group/interview guides with input from her thesis academic advisory committee (AAC). Audio/video recordings were completed using Microsoft Teams, with a digital audio recorder and written notes for backup. All focus groups, interviews, and client and caregiver feedback sessions were transcribed verbatim, except for one virtual interview which utilised detailed notes and member checking to capture information due to technology problems. The PAG, AAC, and clinician feedback sessions were captured through a combination of verbatim transcription, detailed summaries, and/or key point format. Pseudonyms were used for all participants. Table 3 summarises participant participation and role within the focus groups and feedback sessions.

2.5 | Data Collection and Analysis

The primary data sources for this study included PAG meeting notes, AAC notes, notetaker and researcher field notes/reflexive journal, and transcripts. Qualitative data were analysed using the six-phase reflexive thematic analysis approach of Braun and Clarke (2006, 2021). The MFPFE-HHC framed the categories for deductive data analysis, and codes were established inductively through line-by-line coding. Themes were predominantly semantic in nature but included an element of interpretation. An iterative participatory process of research, reflection, and action cycles of data analysis during the feedback sessions and PAG meetings (see Table 3) created the final themes. Data were managed using NVivo 12 qualitative data management software (QSR International n.d.). Study rigour was increased by triangulation of primary data sources, audio/visual recordings, verbatim transcription, peer debriefing, and member checking (Creswell and Creswell 2018; Lincoln and Guba 1985).

In addition, each participant received a follow-up feedback survey after their individual or group interview to capture their experience of engagement. The surveys were brief and developed in the Qualtrics software programme (<https://www.qualtrics.com>) using accessible design features (Stancliffe et al. 2014). Surveys included three (client) or four (caregiver) short multiple-choice questions and an opportunity to add comments (LaDonna et al. 2018). Survey data, both qualitative and quantitative, informed the findings of the qualitative inquiry.

3 | Results

Four themes emerged from the data. Each of the themes – Collaboration, Communication, Empowering Environment, and Tailored Service – held practical engagement strategies for individual-level and programme-level services, with broader implications for policy-level change. This section provides a description of each theme, with sub-themes highlighted in italics. Table 4 provides an amalgamated summary of the practical strategies that emerged relative to each theme.

3.1 | Collaboration

Collaboration was a fundamental feature leading to a sense of engagement. Client and caregiver participants spoke to the *value*

TABLE 2 | Participant demographics for the participatory action study.

Participant category	Total	Age range	Gender			Two-county area		
			W	M	Pass	Clinic 1	Clinic 2	Clinic 3
Clients	11	22–51 Undisclosed ^a	6	4	1	6	2	3
Informal caregiver	6	51–70	6	0	0	2	2	2
Formal caregiver	7	25–64	6	1	0	4	2	1
Clinical staff	7	—	—	—	—	—	—	—

Note: PAG member data included. Age, gender, and two-county area were not obtained for clinical staff to preserve anonymity.

Abbreviations: Dash (—) = not obtained; M = man; Pass = I prefer not to disclose information concerning my gender; W = woman.

^aOne client participant did not disclose his or her age.

TABLE 3 | Participant participation and role in focus groups and feedback sessions.

Project advisory group				
• Membership: Two clients, two caregivers (one formal and one informal). • Met monthly to advise on aspects of the study. • Participated in respective client and caregiver pilot focus groups (included in the final data).				
Key informants for virtual focus groups and interviews				
• Two homogenous groups of clients and caregivers (mix of informal and formal) were created. • Length of focus groups/interviews: 1 hour for clients, and 1.5 hours for caregivers.				
Client focus group/ interview	Number participants	Caregiver focus group/interview	Number participants	
FG 1 – Client	2 (pilot)	FG 1 – Caregiver	2 (pilot)	
FG 2 – Client	2	FG 2 – Caregiver	4	
FG 3 – Client	2	FG 3 – Caregiver	3	
FG 4 – Client	2	FG 4 – Caregiver	3	
Interview 1 – Client	1	Interview 1 – Caregiver	1	
Interview 2 – Client	1			
Interview 3 – Client	1			
Key informants for virtual feedback sessions				
• Reviewed draft themes to gather initial impressions and areas for refinement. • Respective feedback sessions with PAG, AAC, caregivers, clients and study setting members.				
Sequence of feedback sessions				
• First author completed reflexive thematic analysis. Create Draft 1 of CCE document structure. • Feedback session with PAG. Create Draft 2 of CCE document structure. • Respective feedback sessions with AAC, caregiver key informants, client key informants, and PAG. Create Draft 3 of CCE document. • Feedback session with the study setting. • Finalize CCE structure based on feedback.				
FB PAG participants	FB AAC participants	FB caregiver participants	FB client participants	FB study setting participants
FG 1 – Client	AAC 1	FG 2 – Caregiver	FG 3 – Client	MTG 1 – 7 attended
FG 1 – Client	AAC 2	FG 2 – Caregiver	FG 4 – Client	MTG 2 – 1 attended
FG 1 – Caregiver	AAC 3	FG 3 – Caregiver		
FG 1 – Caregiver				

Abbreviations: AAC, academic advisory committee; CCE, client and caregiver engagement; FB, feedback session; FG, focus group; MTG, meeting; PAG, project advisory group.

Abbreviations: AAC, academic advisory committee; CCE, client and caregiver engagement; FB, feedback session; FG, focus group; MTG, meeting; PAG, project advisory group.

of collaboration at both the individual and programme levels of service. Regardless of the format, purpose, or size of the group, the main factor was working together to reach a mutual goal or resolve an issue. Engaging with others provides opportunities to share ideas and experiences, discuss strategy, and learn new information. Lisa, an informal caregiver, highlighted the effectiveness of collaboration at the programme level, stating:

I'll be honest I completed that survey once, and I didn't feel it was an effective way to be part of, say, creating programmes, or I felt that the face-to-face meeting was

where we brainstormed and was more open-ended ... and I had a lot more opportunity to participate.

Collaboration was not always recognised as an in-person activity. Helen, a formal caregiver noted that: 'even when we're not in person we're still working as a team to resolve an issue through email or through a video chat and that's very important to know that I can count on that.'

Caregivers described how *collaboration improves client care*, noting that integrated discussions with the health professionals, client, and their support team produce stronger outcomes.

Of note was the need for inter-disciplinary collaboration between clinical team members, as well as between the clinical team and external health professionals. Involving all necessary individuals removes assumptions, improves understanding, determines level of desired engagement, and supports movement towards a common goal. Understanding the role of

the larger support team needs to begin early, ideally during the intake process. Barb, a formal caregiver, suggested early involvement of the family doctor, stating: 'I think part of the intake for [the dual diagnosis service] should actually be to talk to the family doctor'. Grace, a clinician, spoke to the frustration shared by clients and families when agencies do not

TABLE 4 | Practical engagement strategies for clients and caregivers.

Individual level of service
Engage early and inclusively • Greet/welcome people by name when they enter the clinic/office/appointment regardless of level of communication ability. • Orient people to the office/clinic environment (e.g., magazines, washrooms). • Acknowledge each person in the meeting by completing introductions (client driven). • Identification of caregivers and larger support team early in care and facilitate conversations with client/caregivers to determine who they want/need to attend appointments. • Role clarification for everyone involved in the client's care. • Engage client and caregiver early in care with a conversation to determine desired level of engagement. • Explore client/caregiver expectations of service and communicate what the clinical team/service can offer. • Explore the client and caregiver/family framework or model of care at the start of service. • Spend time to learn about the client to support the building of a trusting relationship. Find out what aspect of the relationship is important to the client (e.g., social aspect or specifics about clinical care). • If possible, maintain established relationships for the duration of care. • Collaboration in several areas of care: intake meeting, care planning, reflection meetings/check-ins, clinical appointments. May include health professionals within or outside the clinical team, caregivers, and other organisations when appropriate. • Offer various methods for collaboration: face-to-face, virtual, phone, email, letter. Appointments/clinical care • Provide a choice of clinician, when applicable. • Empower each person at the table to speak. Speak in order of client, informal caregivers, and formal caregivers. Consider appointment format (e.g., client appointment alone or portion alone, longer appointment). • Ask each person at the table client related health care goals/concerns (in order of client, informal caregiver, formal caregiver). • Collaborate with client/caregiver to create tailored and innovative care plan to meet the client/caregiver needs (e.g., tailored goals, use of pictures in the care plan). • Learn about the uniqueness of the client and then offer tailored therapeutic approach, treatment, and recommendations. • Demonstrate that client/caregiver ideas matter (e.g., talk about their ideas, acknowledge and/or give feedback on their ideas). • If client attends appointment alone, identify client-trusted contact to share pertinent follow-up information. • Offer choice of appointment location such as clinic, community, or home. Consistency might be important for certain clients. • Meet the client where they are on a specific day in terms of their mental health and work from there. • Review key points from previous meeting and gather update at the start of the meeting. • Provide summary of appointment (e.g., main topics discussed, recommendations, medication side-effects, homework, and date of next appointment). Provide accessible summary for client. Designate a scribe. • Share pertinent information with others as necessary. Review limits of confidentiality (e.g., progress note, assessment report, or prescription). • Respect one's time by preparing for the appointment, reading information provided to the clinical team, and ensuring the clinical appointments start on time. Clearly communicate cancellations. • Book next appointment at the end of current appointment. Include the client in the appointment booking. • Caregiver to keep observational notes between appointments and send to clinical team prior to appointment. • Reflection meeting/check-in to discuss recommendations, progress, and next steps. • Consider any limitations of person, role and situation impacting client care when providing recommendations. • Provide positive feedback and reinforcement to clients/caregivers to demonstrate recognition and appreciation for their efforts. • Seek input from client/caregiver first, discuss differing opinions and come to consensus, if possible. Access and availability of the clinical service • Annual review with clients who have been with the service for an extended period. • Provide flexibility in the timing of services (e.g., length appointment, timing of follow-up, participation in groups). • Regular review of wait lists for triage of service needs (clinician driven). • Offer ability for the client/caregiver to initiate a check-in when they are experiencing a challenging moment. • Create a plan for client/caregivers to seek support during a crisis, outside of office hours, and between appointments.

(Continues)

TABLE 4 | (Continued)

Programme level of service	
Feedback as a method of communication	- Consider multiple formats/methods for clients and caregivers to provide feedback. Consider diverse skills/abilities and comfort level. For example, face-to-face/paper/electronic, words/pictures, independent/supported, and group/individual. - Feedback options: exit survey or interview; pre/post survey; ongoing feedback (e.g., 6-month check-in), on-line/physical drop-box or another method to voice concerns, provide input, and communicate with the clinical service. - Have a two-prong approach to capture both client and caregiver feedback. Giving feedback about experiences and services in a format that promotes reflection and discussion. - Consider anonymity when designing programme-level activities aimed at gaining feedback about the service to respect variations in comfort level.
Programme-level considerations	- Ensure a clear purpose and goal for programme-level activity. - Allow everyone a chance to speak. - Demonstrate that client/caregiver ideas matter (e.g., acknowledge and talk in further detail about their ideas). - Mine the wealth of experience from clients and caregivers. - When organising an activity or event it is important to consider appropriate membership, respect availability of members, permissions needed to attend, frequency of meetings, meeting location, purpose and goal of the group, and the impact of the information. - Build a sense of camaraderie within any group that is formed. - Ensure the activity is an agent of change, rather than focus on the format of the programme-level activity (e.g., survey, reflection meeting, focus group, advisory group/council, support group). Could be a group with clients, caregivers, or both. - Have an advisory council with clients when creating materials. - Information booklet for clients/caregivers about the clinical service. - Provide education sessions for staff/client/caregiver on communication skills. - Share information about other community and health services through the organisation's website or in a booklet/pamphlet. - Newsletter from the clinical service to communicate, including questions for the client to answer. - Offer various opportunities to bring people together to discuss ideas. Could be a specific task or bringing a group of clients/caregivers together to discuss what would be helpful for them. - Programme changes/improvements, edit/create materials, increasing awareness of mental health in the community.
Individual and programme levels of service	
Strategies/methods of communication	- Use excitement in your voice to show interest and encouragement. - Support inclusion of client in discussions and decision-making regarding their health care by offering accommodations/strategies: e.g., plain language, list of options, resources to read, review of risks/benefits, and option of caregiver support to facilitate client in understanding/sharing information. - Speak directly to the client to promote a sense of individuality and inclusivity. - Ensure reciprocal conversations to promote discussion, brainstorming and sharing of ideas and opinions. - Use accessible language that is plain, clear, simple, and understood by every person. Adapted and accessible supports comprehension. - Provide clear explanation, do not use technical or medical language, and provide example to facilitate understanding and comfort level. - Use other communication strategies tailored to client (e.g., pictures, fun and interactive activities, slow pace, adaptive equipment). - Create an inclusive environment that promotes utilisation of communication strategies/methods that meet client and/or caregiver needs (e.g., appointment length to allow necessary processing time, consider appointment location). - Use various methods to communicate such as phone, email, written, verbal, pictures, sign language, and interpreter. - Repeat back and clarify to ensure comprehension. - Use non-verbal behaviours to show you are listening such as smiling, head nodding and eye contact. - Caregivers to use clinician picture business cards as a communication tool with the client.

talk to each other. One formal caregiver highlighted the negative impact on client care when left out of care planning discussions. Linda, a formal caregiver, noted:

... there's all these conversations that are being had and we're kind of on the outside, I don't know if

anyone really knows what's going on, but if it's not our decision it's still nice to know we have a plan if things go south.

Clients identified how collaborating with their clinician provided an opportunity to share ideas, increase clarity, validate

client ideas, and reach goals. Sophia, a client, noted: ‘... it’s important for you and for the other person, like the clinician or caregiver, someone just to know about your goals just so you can both work on reaching that goal together’. Jason, a client, shared how collaboration is helpful during his clinical appointments. He stated:

I’m usually saying my things, and I have somebody to bounce feedback off of. I can give them my ideas, and they throw back their opinions and thoughts, and then, basically, we gradually work together to come up with new thoughts or ideas about things.

3.2 | Communication

Communication in its many forms emerged as a fundamental feature of feeling and being engaged. Clients and caregivers were clear about the need to *be heard and have a voice of influence*. Participants described a sense of increased engagement when having opportunities to talk and share information, emphasizing that this information needs to be taken seriously, considered accurate and not disregarded. Caregivers spoke of an inclusive space where people are empowered and supported to speak. Some clients described the capability to speak for themselves and articulate their unique needs and goals, whereas others welcomed or required support. Emma, a client, shared that her mother often provides support by providing important medical history on her behalf. Not being heard can lead to disengagement, frustration, and inadequate help. Nancy, an informal caregiver shared, ‘So if you’re not listening to me and you’re not hearing me you’re not getting us to that step of getting [my son] better’. Jack, a client, described a sense of inclusion when he was able to ‘get [his] feelings out’ and when ‘people are listening to [him]’.

Client and caregiver participants also expressed the need for their voice to have influence or impact. Clients often saw themselves as most knowledgeable about their feelings and experiences and voiced the desire to make decisions about their health care. They felt this showed respect, promoted engagement, and created a sense of control. Some clients required support from a caregiver or health professional to fully understand a decision; others wanted independence in making decisions. Dave, a client, clearly articulated the need to have a voice as he stated: ‘... it’s not a one-sided conversation, not just someone telling me what to do and what’s going to happen, I have some input’.

Participants communicated the need for *effective and clear team-based communication* around clinical service operations, client care, and programme-level activities. This included what is being sought from the mental health service, client and caregiver concerns, and discussions around recommendations for care. Caregivers described how engagement was impacted when they lacked knowledge and clarity about how the clinical service operated. Tina, an informal caregiver, shared:

... having a preliminary meeting where you are clear about what [the service] can offer, how it will look;

that would have been especially helpful for our family since we had no idea where we were going with this or what it was all about, or what to expect.

Caregiver participants further discussed the need for a clinical team to share information around a client’s health care with the larger support team to ensure good communication and continuity of care. Clients appreciated the respect shown when health professionals provided information directly to them, not just to their caregiver.

At the programme level, effective and clear communication was reported to support engagement and drive change. Caregivers spoke of the importance of having clarity on the objective of a programme-level activity. Meaningful engagement was equated with ideas being heard, respected, and utilised; the method or activity were secondary. Susie, an informal caregiver, shared, ‘if you didn’t allow it to be tokenism, there could be a lot of really good things happen’. Kate, a client, stated their feedback could ‘show areas where [the clinical service] need help, and areas that [the clinical service] don’t’.

3.3 | Empowering Environment

Participants were clear how an *environment that is comfortable, inclusive, and supportive* encouraged engagement. Caregivers described experiences where a safe and comfortable space supported dialogue, information sharing, advocacy, and discussion of ideas at a level that may not have occurred otherwise. Ted, a client, spoke about being comfortable when things were explained, stating ‘there have been medication changes, so kinda talking about it and explaining some of the medication and all that, made me feel comfortable ... I had a big relief that I [knew] what they were doing and what they were talking about’. Jason shared how the secretary made him feel comfortable when he entered the office:

Basically [the secretary] would talk to you and ask you how your day is, how it is going, and try to strike up a little bit of a conversation with me and explain to you a little bit about what’s going on and who’s going to meet with you ... make you feel positive about the whole situation.

Inclusive engagement is essential throughout the entire health care experience and needs to be initiated early in the client’s care. The role and degree of engagement are dependent on the circumstance, who comprises the care team, and an individual’s desire and ability to participate.

Clients and caregivers also spoke to a sense of engagement when feeling cared for and supported. For clients a supportive experience resulted when the focus was on their needs and goals and not their disability. Caregivers described a supportive experience as when the health professional took actions to address and resolve client care concerns. Marie, an informal caregiver shared, ‘every time I have ever called [the office, my concern] has been resolved one way or another by the end of the day’. Conversely, some caregivers described a lack of support when their concerns

were not heard or understood, resulting in frustration, poor client care, and safety concerns.

Participants highlighted the importance of establishing a *trusting relationship* from the beginning of service. Client and caregiver participants both recognised that when mutual trust was established between the client/caregiver and health professional, engagement improved. A client participant described feeling comfortable sharing information with the health professional due to the trust they had built, knowing that information given would not be shared with others. Establishing relationships was also noted by caregivers as an important foundation for group activities. Tina, spoke to the importance of ‘build[ing] a sense of camaraderie and mutual goals and then [the group] can get down to work, but unless the group feels comfortable together and then come to understand each other you won’t get very far’.

Participants identified one fundamental factor for any level of engagement: respect. Clients and caregivers alike spoke of the need to have *respect for the person, their role, and the situation*. Clients emphasised the need to be acknowledged as an independent person, regardless of whether they attended the appointment alone or with support. Dave articulated this concept as he stated the importance of the clinical team to ‘think of [him] as an independent person instead of just a thing or extension of [his] parents’. Caregivers also highlighted the centrality of their role in client care, noting that limitations in skills and lack of knowledge concerning the condition, financial resources, and available equipment are detrimental to client success. Linda shared how having limited adaptive equipment in the group home impacted staff’s ability to provide essential client care. At the programme level, caregivers identified that extremely busy caregiving roles may prevent a consistent routine needed to support regular programme-level engagement.

3.4 | Tailored Service

The nature of the service delivery itself and the way it is tailored to presenting needs was the final theme identified. At the individual level of service, clients and caregivers expressed the importance of *person-centred care to meet the unique needs of each person*. Participants highlighted the importance of identifying what they wanted from the clinical service and receiving tailored health care. Alice, an informal caregiver, noted the importance of a health professional addressing her son’s unique needs:

... [my son] is different; he has a very rare syndrome. ... so yeah, we do have to look specifically at the person that you’re dealing with and try to figure out that person’s treatment, not the standard treatment for people that are developmentally delayed.

While client and caregiver interest in programme-level engagement varied in terms of the purpose, approach, and method, participants were clear that obtaining input from service users was important as ‘the knowledge is in your client base and the

people who support them’ (Tina). Although there was no consensus on a single best method of collecting feedback (e.g., direct conversations, surveys, individual or group interviews), the important aspect was to tailor the feedback method to match an individual’s abilities, comfort, and/or preferences. Jason shared his preference, stating:

I think a face-to-face would be good. ... when you do either a survey or questionnaire, you only have a limited space, but when you’re actually talking to someone face-to-face you can basically give them a deeper insight into how you’re feeling or what you are thinking.

Although the potential benefit of client and caregiver engagement was recognised, hesitancy existed about making assumptions that caregiver engagement would improve services or make the lives of service users better.

Access and availability of the clinical service was a key component in achieving tailored service. Participants spoke about feeling engaged when they knew they could connect with the clinical team as needed, such as when there was a crisis, a time-sensitive concern, a question/concern about a clinical recommendation, and/or if they required a quick re-admission. This concept was extended to the ability to connect with another clinical team member when their main contact was out of the office. When a lack of support occurred during a crisis or outside of office hours, the caregivers described feeling alone, struggling to cope, and unaware of where to get help.

4 | Discussion

This study adds an important addition to the literature by exploring client and caregiver engagement within a specialised community-based mental health service for adults with a dual diagnosis, and by way of providing a series of practical strategies at the individual and programme levels of service. By bringing together the unique perspectives of clients, informal caregivers, and formal caregivers, this study acknowledged that system gaps for dual diagnosis service delivery require strong and innovative ways to collaborate, coordinate and integrate service to maximise supports, build capacity, and improve continuity of care. These findings align with core principles articulated by local and regional health authorities, including the importance of integration within and across Ministries (Community Networks of Specialised Care [n.d.](#); KPMG 2012; Ontario Ministry of Health and Long-Term Care and Ministry of Community and Social Services 2008).

4.1 | Themes and Practical Strategies

The four themes emerging from the data – Collaboration, Communication, Empowering Environment, and Tailored Service – were applicable to both individual and programme levels of service and translate into core principles of engagement. Although each theme offered independent engagement strategies, there appeared to be interconnectedness across

themes and strategies. For example, creating a comfortable space that promotes engagement could be achieved by greeting people when they enter the clinic and orienting them to the space. This requires respect and clear communication, demonstrating interaction between the empowering environment and communication themes. The importance of engaging clients with a dual diagnosis and/or their caregivers in care delivery is documented within the literature (e.g., Nicholas et al. 2017; Hall et al. 2009; Rao and Markar 2006); however, this study builds on existing evidence by providing practical strategies for when and how to engage both a client and caregiver within a specialised community-based dual diagnosis service, at individual and programme levels of health care. The practical strategies from this study, although focused on a specialist community team, could potentially support the practical application of the core attributes articulated by Weise et al. (2017, 2018) for generalist mental health professionals working with individuals with a dual diagnosis.

Beyond any associated staff/service user training, many of the practical strategies emerging from this study (see Table 4) do not have large financial or resource implications and do not require complex organisational change. Simple actions, such as having the client introduce who they brought to their appointment and speaking directly to the client, have the potential to create an empowering and engaging service experience while remaining cost neutral. Other strategies may require resource and financial adjustments, such as lengthening appointments to meet communication needs, which in turn may impact the number of people seen by the service. Similar findings on the variation in resource implications of practical strategies were found by Karpusheff et al. (2020) when exploring best practices for including individuals with intellectual disabilities in guideline development for health and social care.

4.2 | Culture of Service Delivery

Clients and their caregivers were clear in their desire to be engaged in the client's health care. Engagement needs to start early and continue throughout the duration of service. The study findings suggest that for a community-based dual diagnosis team, engagement is achieved through building a culture of service delivery that values, encourages, and supports client and caregiver engagement. Part of creating a service delivery culture of this type is creating an inclusive and supportive environment that fosters comfort, trust, and respect. These findings align with research by Newman et al. (2023) who highlighted the need to foster respectful relationships and inclusive environments when supporting an individual with a dual diagnosis.

Although not specific to dual diagnosis, this idea of creating a health care culture that supports engagement can be found within the literature through discussion of topics such as organisational vision, policies and procedures, quality and safety of care, access, training, and service user and health professional attitudes (e.g., Chenoweth and Clements 2011; Graffigna 2017; Ortiz Halabi et al. 2020; Majid 2018). Alignment is also found with previous research on an organisation's cocreation culture in a health care setting (Sharma and Conduit 2016) and resonates with Pizam's (2020) definition of hospitality organisational

culture and conceptual framework. The field would benefit from further research to explore how the themes generated from this study intersect to build a culture of service delivery that values, encourages, and supports client and caregiver engagement.

4.3 | Individuality

Individuality was another concept of achieving meaningful participation identified from this study. Clients were clear they wanted to be recognised and respected as an individual person with value, not an extension of their caregivers. This study indicated that clients seek independence and control over their clinical appointments, but that at times they require support from their caregivers to share pertinent information, understand the conversation, make informed decisions, and remember recommendations. A client's health care support needs are often related to the client's own unique strengths and challenges, capacity, and comfort level. Previous research has highlighted a similar concept, describing frustration experienced by individuals with an intellectual disability when health professionals spoke to their caregivers, rather than speaking directly to them (e.g., Archer et al. 2018; Badcock and Sakellariou 2022).

Another aspect of individuality is client and caregiver preference around the very issue of engagement. A unique addition to the literature is the need to conduct explicit and intentional conversations about engagement preferences with clients and caregivers at the beginning of service. This acknowledges and respects the person, their role, and situation, which in turn supports involvement at a level they feel comfortable with and/or can provide. The impact of attending to engagement preferences should be further assessed, adding to previous research around the support worker role in health support for individuals with an intellectual disability (e.g., Nijhof et al. 2024).

4.4 | Decision-Making

The concept of tokenism versus active and meaningful participation of individuals with an intellectual disability is documented in previous research (Frawley and Bigby 2011; Friedman et al. 2016; Fyson and Fox 2014). This study provides additional evidence that the inclusion of the client and/or caregiver in health care decisions is one way to ensure meaningful engagement in a specialised community-based dual diagnosis service. Creating an inclusive, accessible, and supportive space that meets the unique needs of a client with a dual diagnosis may empower and support engagement in this decision-making process. Supported decision-making is an approach previously used with individuals who have a dual diagnosis (Webb et al. 2020) and resonates with this study as a way to address the unique supports each client may or may not require when making health care decisions. The need for accessible language and materials to support decision-making aligns with previous research on intellectual disability with or without a mental illness (e.g., Hovey and Cheswick 2009; Karpusheff et al. 2020; Newman et al. 2023; Webb et al. 2020). In this study, clients spoke of how involvement in decision-making provided a sense of control over their own care. Hoole and Morgan (2011) found similar results regarding decision-making and power implications in their

research on the engagement of individuals with intellectual disabilities within social services.

4.5 | Communication

This study suggests clear communication may support many aspects of an engaging service experience. At the onset of service, good communication can support the sharing of information about the clinical service, facilitate conversations about client needs, initiate the creation of a trusting relationship, and support effective and tailored care planning. Involving clients, the support team, and relevant professionals removes assumptions, improves understanding, allows role clarity, and supports tailored service. Previous literature has also emphasised the importance of collaboration with clients, caregivers, and support staff during care planning, crisis planning, and discharge planning (KPMG 2012; Weiss and Lunsky 2010), yielding clarity in responsibilities to secure commitment, coordination, and successful outcomes (Haines and Brown 2018). As individuals with a dual diagnosis often require integration of multiple services and providers (Archer et al. 2018; Lunsky et al. 2013; Weiss and Lunsky 2010) understanding the larger support team and their role in the client's care is crucial to inclusive, effective, and efficient care.

Participants within this dual diagnosis setting indicated that they find appointments to be more productive and solution-focused when they are heard and understood. Clients and caregivers should be empowered to use their voice, and this voice needs to be heard and valued, whether at the individual or programme level. These findings are consistent with previous studies that disclosed how clients with a dual diagnosis and their caregivers often feel unheard (Nicholas et al. 2017; Spassiani et al. 2017) and how the stigma experienced by this population often limits their voice (Nicholas et al. 2017). This study builds on existing literature to include perspectives from a community-based dual diagnosis service. This is important, as earlier research has found mental health services are often unable to meet an individual's level of need (Weiss and Lunsky 2010).

4.6 | Accessibility

Barriers to accessing high-quality health care for adults with a dual diagnosis are well documented in the literature (e.g., Man and Kangas 2020; Spassiani et al. 2017; Whittle et al. 2018). This study identified the importance of having an accessible service that supported the engagement of clients and caregivers in times of acute need or crisis. Caregivers emphasised the importance of timely access to a health professional knowledgeable in dual diagnosis, which aligns with past research articulating the barriers created when health professionals lack expertise in dual diagnosis (e.g., Man and Kangas 2020; Whittle et al. 2018). In fact, evidence indicates that many health professionals feel ill-equipped to serve individuals with a dual diagnosis (Ee et al. 2022; Hinde and Mason 2020; Man and Kangas 2020; Whittle et al. 2018).

Weiss and Lunsky (2010) found more conceptually based barriers to service receipt such as negative experiences and lack of trust in professional help. The current study found that having trust in your health professional and feeling valued improved

client and caregiver engagement, but further research is needed to determine the impact of enhanced trust on perceived service accessibility.

5 | Limitations and Recommendations for Further Study

The study setting provides service to people with dual diagnosis within a broad umbrella of conditions that also include learning disability and borderline intellectual functioning, potentially limiting the transferability of the findings to other dual diagnosis settings. Also, this study included only individuals who had communication skills that allowed contributions to group/individual discussions and therefore lacks perspectives of the full range of clients seen in dual diagnosis services. Further research exploring engagement for a broader range of intellectual disabilities is recommended.

The study directly asked about engagement, but many participants lacked experience with this topic and may have struggled to fully conceptualise engagement in practice. As well, the parameters of client and caregiver engagement in this study were limited to individual-level and programme-level services from the perspective of one clinical service. Programme-level exploration was rudimentary in comparison to discussion concerning individual-level service. Further exploration of engagement at various levels is warranted, including organisational and system levels.

The findings expressed by clients and caregivers were similar in their sentiments; however, the small sample size did not allow for separate interpretation of client and caregiver engagement. A larger sample might disclose differences in the power of the themes and engagement strategies across participant categories. Of interest would be whether differences exist between formal and informal caregivers.

6 | Conclusion

This study clearly identified the importance of client and caregiver engagement in a health care setting. The principles and practical opportunities highlighted by this study not only provide a meaningful contribution to a limited body of literature, but they also provide realistic strategies for facilitating meaningful client and caregiver engagement in a community-based dual diagnosis service. These strategies have the potential to create future opportunities that empower and provide a voice to individuals who have historically been marginalised and stigmatised, potentially creating more effective health care for individuals who have a dual diagnosis.

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Conflicts of Interest

At the time of this research study, the first author had a dual role as researcher and occupational therapist at the study setting. To mitigate the potential for coercion, insider bias, and perceived power imbalances the first author was removed from direct involvement in recruitment, transparent about her role to potential study participants during the recruitment process (i.e., written in the letter of information and discussed during the recruitment screening interview) and throughout the project (i.e., discussed at the beginning of the focus group).

Data Availability Statement

Research data are not shared in keeping with our consent agreements.

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